جوستاف روسي انترناشيونال - مصر (EPE) * **طلب علاج أول مرة / متردد**

إسم المريض رباعياً : نيرمين امالي ثابت حنا
المحافظة : القاهره **الحي / المدينة :** البساتين **العنوان :** +++
رقم البطاقة : 28707100104301 **رقم التليفون :** 01200991774 **تاريخ تسجيل الإستمارة :** 17-10-2026
العمر(تاريخ الميلاد) : 10-07-1987 **المهنة :** ربه منزل

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل:-التأمين الصحي-القوات المسلحه-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإخصائي الاجتماعي **المقر بما فيه**
التوقيع:(----------) **التوقيع:**(----------)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)
نتائج الأبحاث : (مرفق)
التشخيص النهائي : سرطان بالثدي (مبادرة)
خطة العلاج : أبحاث أورام ماموجرام وسونار علي الثديين [10888] **قسم العلاج :** الاورام
التعليق على التشخيص : ----------

تقرير اللجنة الثلاثية المتخصصة

الاسم: د/محمد كيلاني **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/ندى حسن **الوظيفة:** ---------- **التوقيع:** ----------
الاسم: د/نرمين رمضان **الوظيفة:** ---------- **التوقيع:** ----------

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقر اللجنة بأن البيانات المدونة فى هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة
رئيس اللجنة و مدير المستشفي.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفي قبل الصرف.

أعضاء اللجنة

ا.د/ ---------- **ا.د/** ---------- **د/** ---------- **د/** ----------

قرار اللجنة

التشخيص النهائي : ----------
خطة العلاج : ----------
المستشفي : ----------**قسم :** ----------
جهة العلاج : ----------**نفقات :** ----------
ملاحظات : ----------

رئيس	مدير عام	ختم
المجلس الطبي المتخصص	مديرية الشئون الصحية	شعار الجمهورية

Name: نيرمين امالي ثابت حنا
National ID: 28707100104301

تقرير طبي

السيد

تحية طيبة وبعد,,

إيماء للطلب المقدم من	**بشأن تقرير طبي للمريض**
بقسم:	**رقم طبي:**

نتشرف بأن نوضح فيما يلي تقريرا عن حالته

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	**تاريخ وساعة الخروج :** PM 01:17 05-07-2026

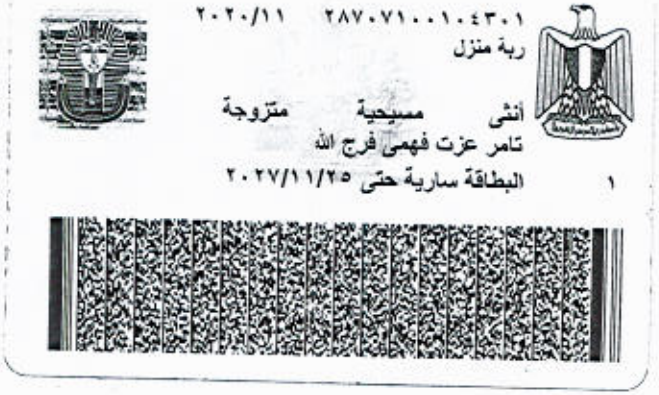
وبعد الكشف والفحوصات والمتابعة وجد انة

المريضة تعاني من ورم غير حميد بالثدى و تحتاج الي **Bilateral mammogram and breast ultrasound**

التاريخ : **05-07-2026**	**الوقت :** **-- --:--**
	الطبيب المعالج : د/شيماء لاشين



National ID – Front
[[DOCTYPE:ID_FRONT]]



National ID – Back
[[DOCTYPE:ID_BACK]]

MDT form

A-Patient ID:
1- Name : ترجمان الماس ثابت حنا
Sex: Female
3- Medical number: 33221549
Old Med :
2-ID number:
Patient Age : 37 Y
2407-2024

0 0

Family history
Phone 01200991774
DM
REASON OF MDT
marital status
NRS Score
GROUP 1
Co morbidity Asthma , 2 cs
Menopausal status
Initial STAGE
Date of MDT 19/10/2024

Clinical data

Breast Imaging :
rt breast UIQ irregular lesion 2 oclock 0.8 x0.1 mm

Mets Workup :
chest x-ray : free
PAUS : free

BX	ER	PR	Her 2 Neu	SISH	KI67
DCIS	8/8				

T	N	M

Sonomammography
cesm: rt breast suspicious nodular enhancement towards retroareolar region
rt axillary LN with increase cortical thickness

PATIENT HISTORY
target u/s FOR Rt UIQ multiple indeterminate nodules to exclude intraductal extension to the nipple
RT BCS+SLNB: free all resected margin (1 mm only)
free LN 0/3

MDT OUTCOME & PLAN

For surgical MDT for wider excision,
PORT, TAM 5 years.

SIGNATURE

MEDICAL ONCOLOGY		RADIOLOGY	
SURGERY		PATHOLOGY	
Psychiatric		Nutrition	

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002824
Patient No.	: 28707100104301	Request Date	: 16/02/2025
Path No.	: 668/25	Verification Date	: 06/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc :		Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
VAGINAL BLEEDING

Nature of specimen
D&C

Gross specimen

Multiple fragmented hemorrhagic pieces measured1x1 cm, totally submitted

Microscopic examination
histopatholgical examination of sections prepared from the endometrial curettings received reveals late secretory glands with exhausted secretory activity. The glands are tortuous and lined by low cuboidal epithelium. The stroma shows late decidualization around well formed spiral arterioles and in between most of the glands. There is no evidence of endometritis or hyperplasia in sections examined.

Diagnosis
Vaginal bleeding.. D&C :
EXAGERATED SECRETORY ENDOMERIUM

Signature
Prof. Dr. Amal Hareedy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13001643
Patient No.	: 28707100104301	Request Date	: 14/08/2024
Lab No.	: 2942/24	Verification Date	: 02/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:
R/O breast suspicious nodular lesion.

Nature of the specimen:
Right wide local excision and sentenil lymph node biopsy specimens , collected at 14/8/2024.

Gross Examination:
Three specimens were recieved :
Sentenil lymph node biopsy:Multiple fibrofatty tissue peices collectivelly measured 3.5x3.5 cm enclosing 3 nodules up to 2x1x0.5cm.
Entire superficial margin:A skin elipse measured 3x1.5 cm with subcutis measured 0.5 cm thick.It showed an incision measured 2.5 cm long.Also received multiple seperate fibrofatty tissue peices measured 3x1x0.5 cm , multisected and totally submitted.
Right wide local excision specimen measured 6x5x2 cm with no covering skin elipse .
Tumor extent:
1- Number: An ill defined firm greyish area (wired) , multisected and totally submitted.
2- Size: 2x1x0.5 cm.
3- Distance from margins:0.2 cm from superior , medial and lateral margins , 0.1 cm from inferior , 2.5 cm from deep and less than 0.1 cm from superficial margin.
Frozen section:
Free SLNs(0/3).
Suspicious superficial margin for DCIS.
Free other margins.
Gross key:
1-4)Grey white area (totally submitted).
5-Medial margin.
6-Lateral margin.

Verified by :
dr.esraa magdy

Printing Date : 22/09/2024 11:11:00 Printing By : Mrs.Mayada Hegazy

CAP
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	الأستاذه / ترمين امالي ثابت حنا .	Request No.	: 13001643
Patien. No.	: 28707100104301	Request Date	: 14/08/2024
Req. No.	: 2942/24	Verification Date	: 02/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

A-Inferior margins.
B-Inferior margin.
C-Superficial margin.
D-Deep margin.
E1,E2-Extra superficial margin.
F3-Sentenil lymph nodes.

Microscopic examination:
Histological type:Ductal carcinoma insitu , low grade.
Intraductal papilloma with pseudoinvasion.
Columanr cell change and ductal hyperplasia.
No evidence of invasion in sections examined.
Angioinvasion: Not detected .
Lymph node involvement:Free SLNs (0/3)
Resection Margins: Free all resected margins.

Diagnosis:
Right breast suspicious nodular lesion, wide local excision and sentenil specimens:
-Ductal carcinoma insitu , low grade.
-Intraductal papilloma.
-Columnar cell changes and ductal hyperplasia.
-No evidence of invasion in sections examined.
-Free all resected margins
-Free SLNs (0/3)
-Stage : pTis , pN0

Prof.Gamal Abdelatty
Dr.Menna Nabil

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 27/06/2024
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID: 28707100104301	Pat. ID: 12175

Combined bilateral breast mammography and complementary ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UIQ irregular mass lesion, of equal density.
- Left breast no suspicious microcalcifications or spiculated masses.
- Complementary ultrasound revealed right breast irregular hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 2 o'clock position measuring 0.8x0.4 mm.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Nonspecific bilateral axillary LNs.

Opinion:

- Right breast UIQ irregular mass lesion ?? benign looking ?? indeterminate for further assessment and close follow up / histopathological correlation if clinically warranted (BIRADS 3)
- Benign findings left breast (BIRADS 2)

Best regards,
Dr/Asmaa Zeinhom, Msc

Not Approved

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 28/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMU[illegible]ISTOCHEMISTRY REPORT

Clinical Data:
Right breast mass.

Nature of Specimen:
Tru-cut biopsy collected 9/7/2024.

Technique:
Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and Hematoxylin as counter stain. The sections treated against:
ER clone (1E2) ,P63(4A4) and Calponin (EP798Y).

Results:
IHC (Positive control showed appropriate reactivity):
P63 and Calponin:Positive in myoepithelial cells outside the ducts.
ER: Strongly positive (8/8).

Conclusion:
Right breast mass, Tru-cut biopsy:
Findings are compatible with intermediate grade DCIS positive for ER (8/8).
Comment:No invasion could be detected within the limits of biopsy of biopsy received.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 07/06/2023
Exam : MAMMOGRAPHY BOTH	Age : 36 Year(s)
Medical ID: 1734	Pat. ID: 12175

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Ultrasound examination using linear probe 6-12MHz

Report:

- ACR C: heterogeneously dense parenchyma.
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Left breast UIQ multiple small well defined hypoechoic nodules area seen at 9o'clcok, 10o'cicok,and 11 o'clcok measuring about 5x5mm, 4x4mm and 9x6mm respectively noted>>> seen with possible ductal communication. It is seen at 3.2 cm distance form the nipple (?? small papilloma ?? Small fibroadenoma)
- Prominent left retro-areolar ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Bilateral mammary fibroadenosis.
- Left breast UIQ probably-benign mass as described (BIRADS 3), for U/S follow up after 6 months

- Signature Dr. Nehal Hamdy
- 07/06/2023 5:01 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]



ERM036729443



National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	: الأستاذة / نرمين امالي ثابت حنا	Request No.	: 13002824
Patient No.	: 28707100104301	Request Date	: 16/02/2025
Path No.	: 668/25	Verification Date	: 06/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
VAGINAL BLEEDING

Nature of specimen
D&C

Gross specimen

Multiple fragmented hemorrhagic pieces measured1x1 cm, totally submitted

Microscopic examination
histopathological examination of sections prepared from the endometrial curettings received reveals late secretory glands w :h exhausted secretory activity. The glands are tortuous and lined by low cuboidal epithelium. The stroma shows late decidualization around well formed spiral arterioles and in between most of the glands. There is no evidence of endometritis or hyperplasia in sections examined.

Diagnosis
Vaginal bleeding.. D&C :
EXAGERATED SECRETORY ENDOMERIUM

Signature
Prof Dr. Amal Hareedy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13001643
Patient No.	: 28707100 04301	Request Date	: 14/08/2024
Path No.	: 2942/24	Verification Date	: 02/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mchamed youss

Pathology Report

7-Superior margins.
8-Inferior margin.
9-Superficial margin.
10-Deep margin.
11,12-Extra superficial margin.
13-Sentenil lymph nodes.

Microscopic examination:
- Histological type:Ductal carcinoma insitu , low grade.
-Intraductal papilloma with pseudoinvasion.
-Columanr cell change and ductal hyperplasia.
-No evidence of invasion in sections examined.
- Angioinvasion: Not detected .
- Lymph node involvement:Free SLNs (0/3)
- Resection Margins: Free all resected margins.

Diagnosis:
Right breast suspicious nodular lesion, wide local excision and sentenil specimens:
-Ducttal carcinoma insitu , low grade.
-Intraductal papilloma.
-Columnar cell changes and ductal hyperplasia.
No evidence of invasion in sections examined.
-Free all resected margins
-Free SLNs (0/3)
-Stage : pTis , pN0

Prof.Gamal Abdelatty
Dr.Menna Nabil

Verified by :
dr.esraa magdy

CAP ACCREDITED
غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Curniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 27/06/2024
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID: 28707100104301	Pat. ID: 12175

Combined bilateral breast mammography and complementary ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UIQ irregular mass lesion, of equal density.
- Left breast no suspicious microcalcifications or spiculated masses.
- Complementary ultrasound revealed right breast irregular hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 2 o'clock position measuring 0.8x0.4 mm.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Nonspecific bilateral axillary LNs.

Opinion:

- Right breast UIQ irregular mass lesion ?? benign looking ?? indeterminate for further assessment and close follow up / histopathological correlation if clinically warranted (BIRADS 3)
- Benign findings eft breast (BIRADS 2)

Best regards,
Dr/Asmaa Zeinhom, Msc

Not Approved

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name	: الأستاذه / ترمين امالي ثابت حنا	Request No.	: 13002824
Patient No.	: 28707100104301	Request Date	: 16/02/2025
Path No.	: 668/25	Verification Date	: 06/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
VAGINAL BLEEDING

Nature of specimen
D&C

Gross specimen

Multiple fragmented hemorrhagic pieces measured1x1 cm, totally submitted

Microscopic examination
histopathoigical examination of sections prepared from the endometrial curettings received reveals late secretory glands with exhausted secretory activity. The glands are tortuous and lined by low cuboidal epithelium. The stroma shows late decidualization around well formed spiral arterioles and in between most of the glands. There is no evidence of endometritis or hyperplasia in sections examined.

Diagnosis
Vaginal bleeding.. D&C :
EXAGERATED SECRETORY ENDOMERIUM

Signature
Prof Dr. Amal Hareedy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 28/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patien:
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMM[illegible]STOCHEMISTRY REPORT

Clinical Data:
Right breast mass.

Nature of Specimen:
Tru-cut biopsy collected 9/7/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2) ,P63(4A4) and Calponin (EP798Y).

Results:
* IPT (Positive control showed appropriate reactivity):
-P63 and Calponin:Positive in myoepithelial cells outside the ducts.
- ER: Strongly positive (8/8).

Conclusion:
Right breast mass, Tru-cut biopsy:
Findings are compatible with intermediate grade DCIS positive for ER (8/8).
Comment:No invasion could be detected within the limits of biopsy of biopsy received.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 07/06/2023
Exam : MAMMOGRAPHY BOTH	Age : 36 Year(s)
Medical ID: 1734	Pat. ID: 12175

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Ultrasound examination using linear probe 6-12MHz

Report:

- ACR C: heterogeneously dense parenchyma.
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Left breast UIQ multiple small well defined hypoechoic nodules area seen at 9o'clcok, 10o'clcok,and 11 o'clcok measuring about 5x5mm, 4x4mm and 9x6mm respectively noted>>> seen with possible ductal communication. It is seen at 3.2 cm distance form the nipple (?? small papilloma ?? Small fibroadenoma)
- Prominent left retro-areolar ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Bilateral mammary fibroadenosis.
- Left breast UIQ probably-benign mass as described (BIRADS 3), for U/S follow up after 6 months

- Signature Dr. Nehal Hamdy
- 07/06/2023 5:01 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

وزارة الصحه و السكان
أمانة المراكز الطبية المتخصصه
مركز أورام دار السلام
"مرمل سابقا"

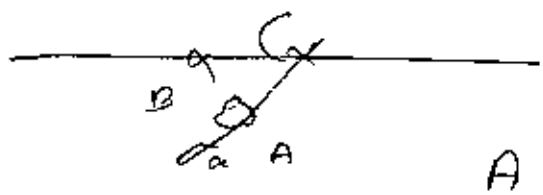
Name. Nermin Amali Thabet	37Y	F
Date: 13/08/2024		

Wire localization for right breast mass:

- Under local anesthesia, antiseptic condition and ultrasound guidance.
  - Ultrasound guided wire localization was inserted into the right breast 2 o'clock lesion
  - Whole distance of the wire till the tip: 2.1 cm
  - Perpendicular distance from skin to tip: 1 cm
  - Distance from perpendicular distance to skin entry: 1.8 cm.
  - Skin mark (X) was placed over the lesion

Best Regards,

Dr. Ahmed M. Rozeka, Msc
Specialist Radiologist



A = 2.1 cm.
B = 1 cm
C = 1.8 cm

Breast Biopsy
[[DOCTYPE:BREAST_BIOPSY]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 15/07/2024
Sex	: Female	Financial Category	: اورام دار لسلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data:
Right breast mass.

Nature of specimen:
Tru cut biopsy, collected at 9/7/2024.

Gross examination:
Three greyish white tissue cores measured up to 1cm , totally submitted.

Microscopic examination:
Examinations revealed cores of breast tissue with many foci of intraductal proliferation showed monotonus cells with moderate pleomorphism and hyperchromasia.

Diagnosis:
Right breast mass, tru cut b opsy:
Findings are compatible with intermediate grade DCIS.
For further IHC (ER,P63,Calponin) to detect any foci of invasion.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 28/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Right breast mass.

Nature of Specimen:
Tru-cut biopsy collected 9/7/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2) ,P63(4A4) and Calponin (EP798Y).

Results:
* IPT (Positive control showed appropriate reactivity):
-P63 and Calponin:Positive in myoepithelial cells outside the ducts.
- ER: Strongly positive (8/8).

Conclusion:
Right breast mass, Tru-cut biopsy:
Findings are compatible with intermediate grade DCIS positive for ER (8/8).
Comment:No invasion could be detected within the limits of biopsy received.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

B.HEYA
Radiology Department

بـ.ـيـة
قسم الأشعة التشخيصية

PATIENT NAME: NERMEEN AMALY THABET HANA
GENDER: Female
DATE : 01-Aug-2024 04:25 PM

PATIENT ID: 10124246439
AGE: 37 y
ORDER: 2970895

TARGET ULTRASOUND EXAMINATION ON LEFT BREAST

INDICATIONS:

- Second look US at the left UIQ tiny enhancing nodule seen in.CESM
- Right breast pathologically proved DCIS

TECHNIQUE:

- Ultrasound examination

REPORT:

Left 9-10:00 well-circusmcribed hypoechoic solid lesion with lobulated outline along with ductal extension measuring 6.9x6.7 mm with anterior ductal extension reaching 13 mm, 22 mm from the nipple, showing no vascularity with color doppler study
Right breast showed 1-3:00 multiple small hypoechoic well-circusmcribed nodules ?? Intraductal extending from the index lesion toward the nipple averaging (4.5 mm)

OPINION:

- **Left 9-10:00 probably benign looking solid lesion ?? Intraductal. Regarding contralateral side, for tissue biopsy to exclude atypical features (BIRADS 3).**
- **Right UIQ multiple indeterminate nodules ?? Intraductal. ?? Satellites ?? Other. For functional study to exclude multicentricity (BIRADS 3)**

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Yasmin Nada, MSc **01-Aug-2024 05:16 PM**

www.Baheya.org
16602
Ultrasound
[[DOCTYPE:ULTRASOUND]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
الأشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 27/06/2024
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID: 28707100104301	Pat. ID: 12175

Combined bilateral breast mammography and complementary ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UIQ irregular mass lesion, of equal density.
- Left breast no suspicious microcalcifications or spiculated masses.
- Complementary ultrasound revealed right breast irregular hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 2 o'clock position measuring 0.8x0.4 mm.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Nonspecific bilateral axillary LNs.

Opinion:

- Right breast UIQ irregular mass lesion ?? benign looking ?? indeterminate for further assessment and close follow up / histopathological correlation if clinically warranted (BIRADS 3)
- Benign findings left breast (BIRADS 2)

Best regards,
Dr/Asmaa Zeinhom, Msc

Not Approved

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
الادارة الاشعة

Patient Name: Nermin Amal Thabet Hanna	Date : 21/11/2022
Exam . ultrasound BREAST (BOTH SIDE)	Age : 35 Year(s)
Medical ID: 2956	Pat. ID: 12175

FOLLOW UP BILATERAL BREAST ULTRASOUND

Report:

As compared to last report dated 30-5-2022:

- Stable size and appearance of the left breast UIQ (11 o'clock) small well defined hypoechoic nodule is seen measuring 8 x 5 mm with possible ductal communication. It is seen at 3.2 cm distance from the nipple (?? small papilloma ?? Small fibroadenoma)
- No right breast solid mass lesions.
- Prominent left retroareolar ducts
- Normal skin thickness, and nipple-areola complex
- Non specific axillary LN

Opinion:

- Left breast UIQ stable probably-benign mass as described (BIRADS 3), for U/S follow up after 6 months.

Signature Dr. Peter Nashat

21/11/2022 11:11 AM

Sonography
[[DOCTYPE:SONOGRAPHY]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Date 30/05/2022	Patient Name: Nermin Amali Thabet Hanna
Age : 35 Year(s)	Exam : MAMMOGRAPHY BOTH
	Medical ID: 889

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Ultrasound examination using linear probe 6-12MHz

Report:

- ACR C: heterogeneously dense parenchyma.
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Left breast UIQ (11 o'clock) small well defined hypoechoic nodule is seen measuring 8 x 4 mm with possible ductal communication. It is seen at 3.2 cm distance form the nipple (?? small papilloma ?? Small fibroadenoma)
- Prominent left retro-areolar ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Bilateral mammary fibroadenosis.
- Left breast U Q probably-benign mass as described (BIRADS 3), for U/S follow up after 6 months.

Signature Dr. Peter Nashat

30/05/2022 01:46 م

Mammogram
[[DOCTYPE:MAMMOGRAM]]

صحة سكان
صحة سكان للأشعة والتحاليل
SEHA SCAN & LAB

LHA
إستشاريين صحة المرأة
LADIES HEALTH ALLIANCE

MRS. NERMIN AMALI THABET
DATE. 07/07/2024
PROF. GAMAL AMIRA
CODE. 264359

US guided skin marker

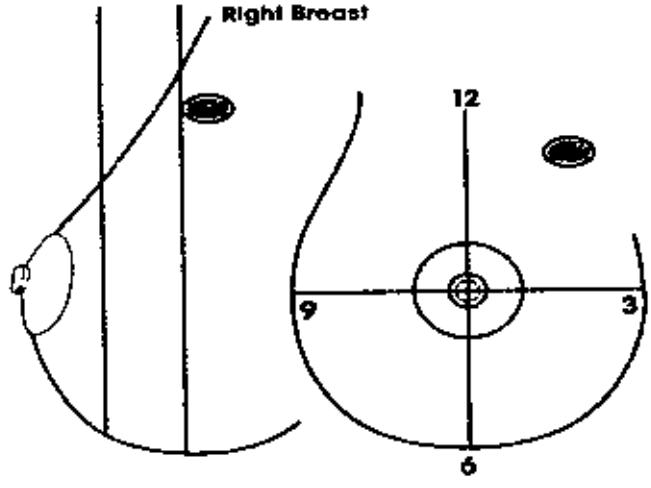
Dear Prof. GamalEmira,

A skin marker was drawn for two tiny rather adherent, 3 mm apart, suspicious solid masses at the upper inner cuadrant (1 O'clock), with the arm elevated in the supine position.

They measure: 5X3 and 3.5X3 mm in diameter and together 8.5X3 mm.

Relation to the Nipple: 10 cm from the nipple.

Depth from skin: 7 mm under the skin.



Prof. Ashraf Selim

N.m

صحتك سعادتك
YOUR HEALTH, YOUR HAPPINESS

Breast Biopsy
[[DOCTYPE:BREAST_BIOPSY]]

38 Mohi El Din Abo El Ezz St., 4th floor, Dokki, Giza, Egypt, TEL.: 02 37 486 100/300/500, FAX: 02 37 49 33 12, www.sohascan.com

صحة سكان
صحة سكان للأشعة والتحاليل
SEHA SCAN & LAB

LHA
إتحاد صحة السيدات
LADIES HEALTH ALLIANCE

MRS. NERMIN AMALI THABET
DATE. 07/07/2024
PROF. GAMAL AMIRA
CODE. 264359

HIGH RESOLUTION ULTRASOUND EXAMINATION OF BOTH BREASTS:

CLINICAL SUMMARY : Bilateral breast lumps.

IMPRESSION :

LEFT BREAST: BIRADS 3 = probably benign.

RIGHT BREAST: BIRADS 4B = moderate suspicion for malignancy.

RECOMMENDATION:-

U/S guided biopsy from the right breast.

Annual screening mammogram starting at the age of 40 years.

Monthly breast self examination. Annual clinical examination.

DETAILED FINDINGS :

TECHNIQUE:

High Resolution Ultrasound with Elastography & Color Doppler.

LEFT BREAST:

- Preserved fibroglandular pattern.
- An oval circumscribed homogenously hypoechoic solid mass with parallel orientation is seen at the upper outer quadrant (1 O'clock). It measures 8X5 mm in diameter. It shows mild posterior enhancement with no vascularity by Power Doppler likely a fibroadenoma. No associated calcifications detected within.
- Normal skin/nipple/areolar complex.
- Preserved axillary lymph nodes.

RIGHT BREAST:

- Preserved fibroglandular pattern.
- Two tiny ill-defined solid nodules are observed at the upper inner quadrant one above the other separated by approximately 3 mm. They measure 5X3 and 3.5X3 mm in diameter. No calcifications or high vascularity detected within.
- Normal skin/nipple/areolar complex.
- Few axillary lymph nodes with thin uniform cortex and preserved fatty hila are noted.

Prof. Ashraf Selim

N.m

صحتك سعادتك
YOUR HEALTH. YOUR HAPPINESS

Sonography
[[DOCTYPE:SONOGRAPHY]]

38 Mohi El Din Abo El Ezz St., 4th floor, Dokki, Giza, Egypt, TEL.: 02 37 486 100/300/500, FAX: 02 37 49 33 12, www.sehascan.com

PATIENT NAME: NERMEEN AMALY THABET HANA
GENDER: Female
DATE : 01-Aug-2024 04:25 PM

PATIENT ID: 10124246439
AGE: 37 y
ORDER: 2970895

TARGET ULTRASOUND EXAMINATION ON LEFT BREAST

INDICATIONS:

- Second look US at the left UIQ tiny enhancing nodule seen in CESM
- Right breast pathologically proved DCIS

TECHNIQUE:

- Ultrasound examination

REPORT:

Left 9-10:00 well-circumscribed hypoechoic solid lesion with lobulated outline along with ductal extension measuring 6.9x6.7 mm with anterior ductal extension reaching 13 mm, 22 mm from the nipple, showing no vascularity with color doppler study
Right breast showed 1-3:00 multiple small hypoechoic well-circumscribed nodules ?? Intraductal extending from the index lesion toward the nipple averaging 4.5 mm

OPINION:

- **Left 9-10:00 probably benign looking solid lesion ?? Intraductal. Regarding contralateral side, for tissue biopsy to exclude atypical features (BIRADS 3).**
- **Right UIQ multiple indeterminate nodules ?? Intraductal. ?? Satellites ?? Other. For functional study to exclude multicentricity (BIRADS 3)**

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Yasmin Nada, MSc **01-Aug-2024 05:16 PM**

www.Baheya.org
16602
٨ شارع ملوية أول شارع الهرم محور دارى . الجيزة
٠ الحي الأول الشيخ زايد امام كلية الهندسة

Sonography
[[DOCTYPE:SONOGRAPHY]]

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

صحة

MDT FORM 8 / 8 / 2024 GB

Name	نرمين امال ثابت	MRN	٢٩٠٧١٢٤
		ID	٢٨٧٠٧٢٠٢٠٠٠٧١
Age	37 yrs	Phone	01206991774
Menopausal status	premenopausal	Marital status	married
NRS SCORE		Family history	uncle & grandfather gastric cancer
BMI		REASON OF MDT	
CO morbidity	Asthma, 2CS		

Clinical Data

BX: DCIS intermediate grade L

ER: 8/8 +ve	PR:	HER 2:	KI67:



.8 x .1 cm

Sonomammography: Rt breast UIQ irregular lesion 20'clock

Mets Workup: Chest X ray free

PAMS free

T1N0M0

Recommendations: 9 pt presented with rt breast mass for 2 months:

CESM: Rt breast suspicious nodular enhancement towards retroareolar region, Rt axillary LN & thoracic cortical thickness

MDT OUTCOME & PLAN: For 2 wires localization → WLE & SLNB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 28/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Right breast mass.

Nature of Specimen:
Tru-cut biopsy collected 9/7/2024.

Technique:
-Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP [illegible]hromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2) ,P63(4A4) and Calponin (EP798Y).

Results:
* IPT (Positive control showed appropriate reactivity):
-P63 and Calponin:Positive in myoepithelial cells outside the ducts.
- ER: Strongly positive (8/8).

Conclusion:
Right breast mass, Tru-cut biopsy:
Findings are compatible with intermeiate grade DCIS positive for ER (8/8).
Comment:No invasion could be detected within the limits of biopsy of biopsy received.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Women's Health Outreach Program

Name: Nermine Amali Sabet Hana
Date: 08-08-2024

Targeted US to Right breast

Clinical Summary: Right sided pathologically proven DCIS

Detailed Findings

Right breast

- **2:00 three small hypoechoic solid nodules each is of average size 3 to 5mm maximum diameter and separated by 3mm apart those are seen 7mm deep to skin. The nearest one to nipple is located at zone b about 28 mm far from nipple. Singie prominent retroareolar duct of 19mm length and reaching 2mm in calibre with no inspissated secretion.**
- No cystic lesions.
- Normal skin/nipple/areolar complex
- No pathological axillary LNs.

Conclusion:

- **Right breast:** Pathological y proven DICS (BI-RADS 6)
- **Left breast:** BIRADS 3 benign solid lesion
-

Much Obliged,
Dr. Rehab Elsheikh , MD

Sonography
[[DOCTYPE:SONOGRAPHY]]

توضيح امام

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name. Nermin Amali Thabet Hanna	Date : 27/06/2024
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID 28707100104301	Pat. ID: 12175

Combined bilateral breast mammography and complementary ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UIQ irregular mass lesion, of equal density.
- Left breast no suspicious microcalcifications or spiculated masses.
- Complementary ultrasound revealed right breast irregular hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 2 o'clock position measuring 0.8x0.4 mm.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Nonspecific bilateral axillary LNs.

Opinion:

- Right breast UIQ irregular mass lesion ?? benign looking ?? indeterminate for further assessment and close follow up / histopathological correlation if clinically warranted (BIRADS 3)
- Benign findings left breast (BIRADS 2)

Best regards,
Dr/Asmaa Zeinhom, Msc

Not Approved

Mammogram
[[DOCTYPE:MAMMOGRAM]]

ده التقرير القديم

B.HEYA
Radiology Department

بهية
قسم الأشعة التشخيصية

PATIENT NAME: NERMEEN AMALY THABET HANA
GENDER: Female
DATE : 31-Jul-2024 03:52 PM

PATIENT ID: 10124246439
AGE: 37 y
ORDER: 2969012,2969504

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:
- right breast DCIS

TECHNIQUE:
- Contrast Enhanced Spectral Mammography CC and MLO views
- Complementary ultrasound

REPORT:
- Bilateral dense fibroglandular densities (ACR C).
- Bilateral scattered homogeneously enhancing nodules likely adenotic
- Right upper inner deeply seated heterogeneously enhancing nodules arranged in linear pattern largest measured 3 mm with anterior enhancing nodular extension towards retroareolar region confirmed by ultrasound
- Left UIQ homogeneously enhancing nodule corresponding by ultrasound to well dfeined hypoechoic lesion
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right axillary lymph node with increased cortical thickness reaching 4 mm.

OPINION:
-Right breast suspicious nodular enhancement towards retroareolar region (BiRADS 6).
- No left breast pathological mass or nor mass enahncement with UIQ small adenoma (BIRADS 3)

Reported By,

Dr. Amira Atef Hussain, MSc **31-Jul-2024 04:32 PM**

www.Baheya.org
16602
مركز بهية الهرم
٨ شارع طوبة أول شارع الهرم بجوار دلوي ، الجيزة
مستشفى بهية الشيخ زايد
١٠ الحي الأول الشيخ زايد امام كلية الهندسة

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
دارة الاشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 28/07/2024
Exam : Ultrasound ABDOMEN& PELVIS	Age : 37 Year(s)
Medical ID: 2407/2024	Pat. ID: 12175

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Mildly enlarged in size (about 18 cm at the right MCL), smooth outline, bright echopattern. No intra hepatic biliary radicle dilatation.*
- *Portal vein is not dilated and patent .*

Gall Bladder: -

- *Average size , normal wall thickness, no stones.*
- *CBD is not dilated.*

Spleen: *Average size (About 12 cm), displaying homogenous echopattern.*

Kidneys: -

- *Are of normal size and site , normal echogenicity , no stone or backpressure changes , no solid or cystic lesion.*
- *Both ureters are noted dilated.*

URINARY BLADDER*: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.*

Uterus: -:

- *anti-verted, average sized uterus. Average endometrial thickness, no focal lesions.*

Ovaries: - *both are average in size with no dominant follicle could be detected.*

- ***No free fluid in the Douglas pouch.***
- *Para-aortic and pancreatic area: no gross lesion*

Opinion:

- *Bright hepatomegaly likely fatty.*
- *Otherwise; normal rest of study.*

Best Regards,
Dr. Maataz Adel, MSc.

Signature Dr . Moataz [illegible]im
PM 2:05 2024/07/28

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Date : 28/7/2024	Patient Name: Nermin Amali Thabet Hanna
Age : [illegible](s)	Exam : X-RAY CHEST
Pat. ID: 12175	Medical ID: 247/2024

Plain X-ray Chest (PA view) revealed:

- *Clear both lung fields.*
- *Clear both costo-phrenic angles.*
- *Normal cardiac size & configuration.*

Best regards

Dr. Moataz Adel ,MSc.

Signature Dr . Moataz Adel Ebranim
28/07/2024 2:09 PM

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

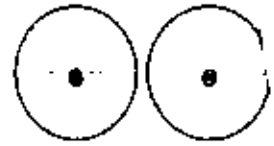
MDT form

A-Patient ID:
1- Name : ترمين امقى ثابت حنا Sex: Female
3- Medical number: 83221549
Old Med : 2407-2024

2-ID number: 28707100104301

Patient Age : 37 Y

MDT



Family history	Family History +ve	Phone	Phone 01200991774	BMI	BMI	REASON OF MDT	REASON OF MDT REMOT
marital status	Marital Status MARRIED	NRS Score	NRS Score	GROUP	GROUP a	Date of MDT	Date Of MDT 08 / 08 / 2024
Co morbidity	Co Morbidity ashtma , 2 cs	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
rt breast UIQ irregular lesion 2 oclock 0.8 x0.1 mm

Mets Workup :
chest xray : free
PAUS : free

BX	ER	PR	Her 2 Neu	SISH	KI67
DCIS BX COMMENT:	8/8 ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT :	

T	N	M

Sonomammography
cesm: rt breast susupicous nodular enahncement towards retroareolar region
rt axillary LN with increase cortical thickness

PATIENT HISTORY
target u/s FOR Rt UIQ multiple indeterminate nodules to exclude intraductal extension to the nipple

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/	RADIOLOGY	RADIOLOGY د/
SURGERY	SURGERY د/	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM / / 20 G

Name	نيرمين احمد ثابت	MRN	٨٣٢٢١٥٤٩
		ID	٠١٢٨٨٨٧٣١١
Age	37	Phone	٠١٢٠٠٩٩١٧٧٤
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity	Asthma		

Clinical Data

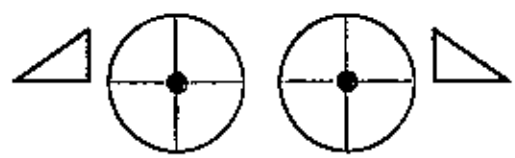
BX: OCIs

ER:	PR:	HER 2:	Ki67:
8/8			

Sonomammography:

Mets Workup:

R L



T N M

Recommendations:

Rt BCS + SNIB Recieved Port and on adj Tam for 5yrs

MRI Pelvis shows Rt adnexal Ovarian mass

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Pres ST.E Program for Women's Health
Ministry of H... and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : ترمين امقى ثابت حنا
Sex: Female
3- Medical number: 83221549
Old Med :
2407-2024
2-ID number:
Patient Age : 37 Y

Family history	+ve	Phone	01200991774	BMI		REASON OF MDT	
marital status		NRS Score		GROUP		Date of MDT	07/11/2024
Co morbidity	asthma , 2 cs	Menopausal status		Initial STAGE			

Clinical data

Breast Imaging :
rt breast UIQ irregular lesion 2 oclock 0.8 x0.1 mm

Mets Workup :
chest x-ray : free
PAUS : free

BX	ER	PR	Her 2 Neu	SISH	Ki67
DCIS	8/8				

T	N	M

Sonomammography
cesm: rt breast suspicious nodular enhancement towards retroareolar region
rt axillary LN with increase cortical thickness

PATIENT HISTORY
target u/s FOR Rt UIQ multiple indeterminate nodules to exclude intraductal extension to the nipple
RT BCS+SLNB: free all resected margin (1 mm only)
free LN 0/3

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
PORT, TAM 5 years.

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psych-Onc	Nutrition	Nutrition

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002824
Patient No.	: 28707100104301	Request Date	: 16/02/2025
Path No.	: 668/25	Verification Date	: 06/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
VAGINAL BLEEDING

Nature of specimen
D&C

Gross specimen

Multiple fragmented hemorrhagic pieces measured1x1 cm, totally submitted

Microscopic examination
histopathlgical examination of sections prepared from the endometrial curettings received reveals late secretory glands with exhausted secretory activity. The glands are tortuous and lined by low cuboidal epithelium. The stroma shows late decidualization around well formed spiral arterioles and in between most of the glands. There is no evidence of endometritis or hyperplasia in sections examined.

Diagnosis
Vaginal bleeding.. D&C :
EXAGERATED SECRETORY ENDOMERIUM

Signature
Prof. Dr. Amal Hareedy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

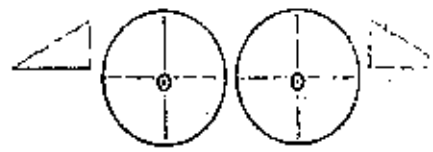
Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : ترمين امانى ثابت حنا
Sex: Female
3- Medical number: 83221549
Old Med : 2407-2024

2-ID number: 28707100104301
Patient Age : 37 Y



Family history	Family history +Ve	Phone	01200991774	BMI		REASON OF MDT	REASON OF MDT
marital status	Marital Status MARRIED	NRS Score		GROUP	GROUP D	Date of MDT	Date of MDT 2025-05-13
Co morbidity	Co Morbidity asthma , 2 cs	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
rt breast UIQ irregular lesion 2 oclock 0.8 x0.1 mm

Mets Workup :
chest x-ray : free
PAUS : free

BX	ER	PR	Her 2 Neu	SISH	KI67
DCIS	8/8				
BX COMMENT:	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT :	

T	N	M

Sonomammography
cesm: rt breast suspicious nodular enhancement towards retroareolar region
rt axillary LN with increase cortical thickness

PATIENT HISTORY
target u/s FOR Rt UIQ multiple indeterminate nodules to exclude intraductal extension to the nipple
RT BCS+SLNB: free all resected margin (1 mm only)
free LN 0/3
on adj tam for 5 years
MRI pelvis show rt adenxal (ovarian) mass
PELVIC MRI RT ADENEXAL LESION 6 X6X 10 CM WITH NO ENHANCED SOFT TISSUE COMP.
CA125 NORMAL

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
FOLLOW UP EVERY 3 MS BY PAUS CYST EXCISION IF SYMPTOMATIC

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13001643
Patient No.	: 28707100104301	Request Date	: 14/08/2024
Path No.	: 2942/24	Verification Date	: 02/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - مباشره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:
Right breast suspicious nodular lesion.

Nature of the specimen:
Right wide local excision and sentenil lymph node biopsy specimens , collected at 14/8/2024.

Gross Examination:
Three specimens were recieved :
-Sentenil lymph node biopsy:Multiple fibrofatty tissue peices collectivelly measured 3.5x3.5 cm , enclosing 3 nodules up to 2x1x0.5cm.
-Extra superficial margin:A skin elipse measured 3x1.5 cm with subcutis measured 0.5 cm thick.It showed an incision measured 2.5 cm long.Also received multiple seperate fibrofatty tissue peices measured 3x1x0.5 cm , multisected and totally submitted.
-Right wide local excision specimen measured 6x5x2 cm with no covering skin elipse .
- Tumor extent
o Number: An ill defined firm grey'sh area (wired) , multisected and totally submitted.
o Size: 2x1x0.5 cm.
- Distance from margins:0.2 cm from superior , medial and lateral margins , 0.1 cm from inferior , 2.5 cm from deep and less than 0.1 cm from superficial margin.
- Frozen section:
-Free SLNs(0/3).
-Suspicious superficial margin for DCIS.
-Free other margins.
- Gross key:
1-4)Grey white area (totally submitted).
5-Medial margin.
6-Lateral margin.

Verified by :
dr.esraa magdy

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

MDT FORM 1 / 10/ 2024 G B

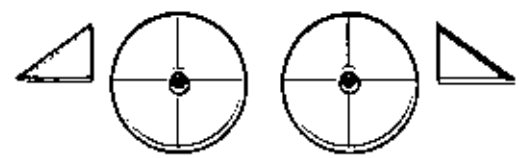
Name	ترمسين نماى ثابت	**MRN**	٨٣٢٢١٠٤٩
		ID	2407 / 2024
Age	37yo	**Phone**	01288017311
Menopausal status	Pre-menopausal	**Marital status**	Married
NRS SCORE		**Family history**	-ve
BMI		**REASON OF MDT**	٠١٢٠٠٩٩١٧٧٤
CO morbidity	BA.		

Clinical Data

BX: DCIS low grade

ER:	PR:	HER 2:	Ki67:

R L



Sonomammography:

Mets Workup:

TNM

Recommendations:

Post IT. BCS. +SLNBx, 14/8/2024.
Pathology free all Resected Margins. DCIS low grade
No evidence of invasion in all Section free SLN (0/3).
Stage pTis, pN0.

MDT OUTCOME & PLAN

SIGNATURE					
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

صحة مصر

MDT FORM 8/8/2024 GB

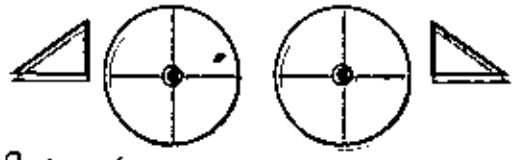
Name	نرمين امالي ثابت	**MRN**	٢٢٠٧١٢٤
		ID	٢٨٥١٠١٠٢٠١٠١
Age	37 yrs	**Phone**	01206991779
Menopausal status	premenopausal	**Marital status**	married
NRS SCORE		**Family history**	uncle & grandfather
BMI		**REASON OF MDT**	gastric cancer
CO morbidity	Asthma, 2CS		

Clinical Data

BX: DCIS intermediate grade

ER: 8/8 +ve	PR:	HER 2:	Ki67:

L



.8 x .1 mm

T1N0M0

Sonomammography: Rt breast UIQ irregular lesion 20'clock

Meta Workup: Chest X ray free

PAUS free

Recommendations:

9- pt presented with rt breast mass for 2 months:

CESM: Rt breast suspicious nodular enhancement towards retroareolar region, Rt axillary LN6 metastasis cortical thickness

MDT OUTCOME & PLAN

For 2 wires localization → WLE & SLNB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : ترمين امانى ثابت حنا
Sex: Female
3- Medical number: 83221549
Old Med : 2407-2024
2-ID number:
Patient Age : 37 Y

MDT

Family history	Family History: +ve	Phone	Phone: 01200991774	BMI	BMI	REASON OF MDT	REASON OF MDT: POST OPERATIVE
marital status	Marital Status: MARRIED	NRS Score	NRS Score	GROUP	GROUP: B	Date of MDT	Date Of MDT: 19/10/2024
Co morbidity	Co Morbidity: asthma , 2 cs	Menopausal status	Menopausal Status: PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
rt breast UIQ irregular lesion 2 oclock 0.8 x0.1 mm

Mets Workup :
chest x-ray : free
PAUS : free

BX	ER	PR	Her 2 Neu	SISH	KI67
DCIS	8/8				
BX COMMENT:	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT :	

T	N	M

Sonomammography
cesm: rt breast suspicious nodular enhancement towards retroareolar region
rt axillary LN with increase cortical thickness

PATIENT HISTORY
target u/s FOR Rt UIQ multiple indeterminate nodules to exclude intraductal extension to the nipple
RT BCS+SLNB: free all resected margin (1 mm only)
free LN 0/3

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN:
For surgical MDT for wider excision,
PORT, TAM 5 years.

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY: د.رضوى حامد	RADIOLOGY	RADIOLOGY: د/شريف سليم
SURGERY	SURGERY:	PATHOLOGY	PATHOLOGY:
Psychiatric	Psychiatric:	Nutrition	Nutrition:



ERM036729443



National ID – Front & Back
[[DOCTYPE:ID_FRONT_BACK]]

Patient Name	: الأستاذة / نرمين امالي ثابت حنا	Request No.	: 13002824
Patient No.	: 28707100104301	Request Date	: 16/02/2025
Path No.	: 668/25	Verification Date	: 06/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
VAGINAL BLEEDING

Nature of specimen
D&C

Gross specimen

Multiple fragmented hemorrhagic pieces measured1x1 cm, totally submitted

Microscopic examination
histopathological examination of sections prepared from the endometrial curettings received reveals late secretory glands w :h exhausted secretory activity. The glands are tortuous and lined by low cuboidal epithelium. The stroma shows late decidualization around well formed spiral arterioles and in between most of the glands. There is no evidence of endometritis or hyperplasia in sections examined.

Diagnosis
Vaginal bleeding.. D&C :
EXAGERATED SECRETORY ENDOMERIUM

Signature
Prof Dr. Amal Hareedy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13001643
Patient No.	: 28707100 04301	Request Date	: 14/08/2024
Path No.	: 2942/24	Verification Date	: 02/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mchamed youss

Pathology Report

7-Superior margins.
8-Inferior margin.
9-Superficial margin.
10-Deep margin.
11,12-Extra superficial margin.
13-Sentenil lymph nodes.

Microscopic examination:
- Histological type:Ductal carcinoma insitu , low grade.
-Intraductal papilloma with pseudoinvasion.
-Columanr cell change and ductal hyperplasia.
-No evidence of invasion in sections examined.
- Angioinvasion: Not detected .
- Lymph node involvement:Free SLNs (0/3)
- Resection Margins: Free all resected margins.

Diagnosis:
Right breast suspicious nodular lesion, wide local excision and sentenil specimens:
-Ducttal carcinoma insitu , low grade.
-Intraductal papilloma.
-Columnar cell changes and ductal hyperplasia.
No evidence of invasion in sections examined.
-Free all resected margins
-Free SLNs (0/3)
-Stage : pTis , pN0

Prof.Gamal Abdelatty
Dr.Menna Nabil

Verified by :
dr.esraa magdy

CAP ACCREDITED
غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Curniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 27/06/2024
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID: 28707100104301	Pat. ID: 12175

Combined bilateral breast mammography and complementary ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UIQ irregular mass lesion, of equal density.
- Left breast no suspicious microcalcifications or spiculated masses.
- Complementary ultrasound revealed right breast irregular hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 2 o'clock position measuring 0.8x0.4 mm.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Nonspecific bilateral axillary LNs.

Opinion:

- Right breast UIQ irregular mass lesion ?? benign looking ?? indeterminate for further assessment and close follow up / histopathological correlation if clinically warranted (BIRADS 3)
- Benign findings eft breast (BIRADS 2)

Best regards,
Dr/Asmaa Zeinhom, Msc

Not Approved

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name	: الأستاذه / ترمين امالي ثابت حنا	Request No.	: 13002824
Patient No.	: 28707100104301	Request Date	: 16/02/2025
Path No.	: 668/25	Verification Date	: 06/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
VAGINAL BLEEDING

Nature of specimen
D&C

Gross specimen

Multiple fragmented hemorrhagic pieces measured1x1 cm, totally submitted

Microscopic examination
histopathoigical examination of sections prepared from the endometrial curettings received reveals late secretory glands with exhausted secretory activity. The glands are tortuous and lined by low cuboidal epithelium. The stroma shows late decidualization around well formed spiral arterioles and in between most of the glands. There is no evidence of endometritis or hyperplasia in sections examined.

Diagnosis
Vaginal bleeding.. D&C :
EXAGERATED SECRETORY ENDOMERIUM

Signature
Prof Dr. Amal Hareedy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 28/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patien:
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMM[illegible]STOCHEMISTRY REPORT

Clinical Data:
Right breast mass.

Nature of Specimen:
Tru-cut biopsy collected 9/7/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2) ,P63(4A4) and Calponin (EP798Y).

Results:
* IPT (Positive control showed appropriate reactivity):
-P63 and Calponin:Positive in myoepithelial cells outside the ducts.
- ER: Strongly positive (8/8).

Conclusion:
Right breast mass, Tru-cut biopsy:
Findings are compatible with intermediate grade DCIS positive for ER (8/8).
Comment:No invasion could be detected within the limits of biopsy of biopsy received.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 07/06/2023
Exam : MAMMOGRAPHY BOTH	Age : 36 Year(s)
Medical ID: 1734	Pat. ID: 12175

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Ultrasound examination using linear probe 6-12MHz

Report:

- ACR C: heterogeneously dense parenchyma.
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Left breast UIQ multiple small well defined hypoechoic nodules area seen at 9o'clcok, 10o'clcok,and 11 o'clcok measuring about 5x5mm, 4x4mm and 9x6mm respectively noted>>> seen with possible ductal communication. It is seen at 3.2 cm distance form the nipple (?? small papilloma ?? Small fibroadenoma)
- Prominent left retro-areolar ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Bilateral mammary fibroadenosis.
- Left breast UIQ probably-benign mass as described (BIRADS 3), for U/S follow up after 6 months

- Signature Dr. Nehal Hamdy
- 07/06/2023 5:01 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

وزارة الصحه و السكان
أمانة المراكز الطبية المتخصصه
مركز أورام دار السلام
"مرمل سابقا"

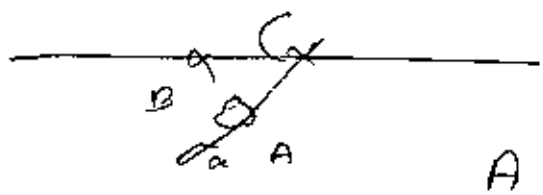
Name. Nermin Amali Thabet	37Y	F
Date: 13/08/2024		

Wire localization for right breast mass:

- Under local anesthesia, antiseptic condition and ultrasound guidance.
  - Ultrasound guided wire localization was inserted into the right breast 2 o'clock lesion
  - Whole distance of the wire till the tip: 2.1 cm
  - Perpendicular distance from skin to tip: 1 cm
  - Distance from perpendicular distance to skin entry: 1.8 cm.
  - Skin mark (X) was placed over the lesion

Best Regards,

Dr. Ahmed M. Rozeka, Msc
Specialist Radiologist



A = 2.1 cm.
B = 1 cm
C = 1.8 cm

Breast Biopsy
[[DOCTYPE:BREAST_BIOPSY]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 15/07/2024
Sex	: Female	Financial Category	: اورام دار لسلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data:
Right breast mass.

Nature of specimen:
Tru cut biopsy, collected at 9/7/2024.

Gross examination:
Three greyish white tissue cores measured up to 1cm , totally submitted.

Microscopic examination:
Examinations revealed cores of breast tissue with many foci of intraductal proliferation showed monotonus cells with moderate pleomorphism and hyperchromasia.

Diagnosis:
Right breast mass, tru cut b opsy:
Findings are compatible with intermediate grade DCIS.
For further IHC (ER,P63,Calponin) to detect any foci of invasion.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 28/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Right breast mass.

Nature of Specimen:
Tru-cut biopsy collected 9/7/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2) ,P63(4A4) and Calponin (EP798Y).

Results:
* IPT (Positive control showed appropriate reactivity):
-P63 and Calponin:Positive in myoepithelial cells outside the ducts.
- ER: Strongly positive (8/8).

Conclusion:
Right breast mass, Tru-cut biopsy:
Findings are compatible with intermediate grade DCIS positive for ER (8/8).
Comment:No invasion could be detected within the limits of biopsy received.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

B.HEYA
Radiology Department

بـ.ـيـة
قسم الأشعة التشخيصية

PATIENT NAME: NERMEEN AMALY THABET HANA
GENDER: Female
DATE : 01-Aug-2024 04:25 PM

PATIENT ID: 10124246439
AGE: 37 y
ORDER: 2970895

TARGET ULTRASOUND EXAMINATION ON LEFT BREAST

INDICATIONS:

- Second look US at the left UIQ tiny enhancing nodule seen in.CESM
- Right breast pathologically proved DCIS

TECHNIQUE:

- Ultrasound examination

REPORT:

Left 9-10:00 well-circusmcribed hypoechoic solid lesion with lobulated outline along with ductal extension measuring 6.9x6.7 mm with anterior ductal extension reaching 13 mm, 22 mm from the nipple, showing no vascularity with color doppler study
Right breast showed 1-3:00 multiple small hypoechoic well-circusmcribed nodules ?? Intraductal extending from the index lesion toward the nipple averaging 4.5 mm

OPINION:

- **Left 9-10:00 probably benign looking solid lesion ?? Intraductal. Regarding contralateral side, for tissue biopsy to exclude atypical features (BIRADS 3).**
- **Right UIQ multiple indeterminate nodules ?? Intraductal. ?? Satellites ?? Other. For functional study to exclude multicentricity (BIRADS 3)**

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Yasmin Nada, MSc **01-Aug-2024 05:16 PM**

www.Baheya.org
16602
Sonography
[[DOCTYPE:SONOGRAPHY]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
الأشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 27/06/2024
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID: 28707100104301	Pat. ID: 12175

Combined bilateral breast mammography and complementary ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UIQ irregular mass lesion, of equal density.
- Left breast no suspicious microcalcifications or spiculated masses.
- Complementary ultrasound revealed right breast irregular hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 2 o'clock position measuring 0.8x0.4 mm.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Nonspecific bilateral axillary LNs.

Opinion:

- Right breast UIQ irregular mass lesion ?? benign looking ?? indeterminate for further assessment and close follow up / histopathological correlation if clinically warranted (BIRADS 3)
- Benign findings left breast (BIRADS 2)

Best regards,
Dr/Asmaa Zeinhom, Msc

Not Approved

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
الادارة الاشعة

Patient Name: Nermin Amal Thabet Hanna	Date : 21/11/2022
Exam . ultrasound BREAST (BOTH SIDE)	Age : 35 Year(s)
Medical ID: 2956	Pat. ID: 12175

FOLLOW UP BILATERAL BREAST ULTRASOUND

Report:

As compared to last report dated 30-5-2022:

- Stable size and appearance of the left breast UIQ (11 o'clock) small well defined hypoechoic nodule is seen measuring 8 x 5 mm with possible ductal communication. It is seen at 3.2 cm distance from the nipple (?? small papilloma ?? Small fibroadenoma)
- No right breast solid mass lesions.
- Prominent left retroareolar ducts
- Normal skin thickness, and nipple-areola complex
- Non specific axillary LN

Opinion:

- Left breast UIQ stable probably-benign mass as described (BIRADS 3), for U/S follow up after 6 months.

Signature Dr. Peter Nashat

21/11/2022 11:11 AM

Sonography
[[DOCTYPE:SONOGRAPHY]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
إدارة الأشعة

Date 30/05/2022	Patient Name: Nermin Amali Thabet Hanna
Age : 35 Year(s)	Exam : MAMMOGRAPHY BOTH
	Medical ID: 889

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Ultrasound examination using linear probe 6-12MHz

Report:

- ACR C: heterogeneously dense parenchyma.
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Left breast UIQ (11 o'clock) small well defined hypoechoic nodule is seen measuring 8 x 4 mm with possible ductal communication. It is seen at 3.2 cm distance form the nipple (?? small papilloma ?? Small fibroadenoma)
- Prominent left retro-areolar ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Bilateral mammary fibroadenosis.
- Left breast U Q probably-benign mass as described (BIRADS 3), for U/S follow up after 6 months.

Signature Dr. Peter Nashat

30/05/2022 01:46 م

Mammogram
[[DOCTYPE:MAMMOGRAM]]

صحة سكان
صحة سكان للأشعة والتحاليل
SEHA SCAN & LAB

LHA
إستشاريين صحة المرأة
LADIES HEALTH ALLIANCE

MRS. NERMIN AMALI THABET
DATE. 07/07/2024
PROF. GAMAL AMIRA
CODE. 264359

US guided skin marker

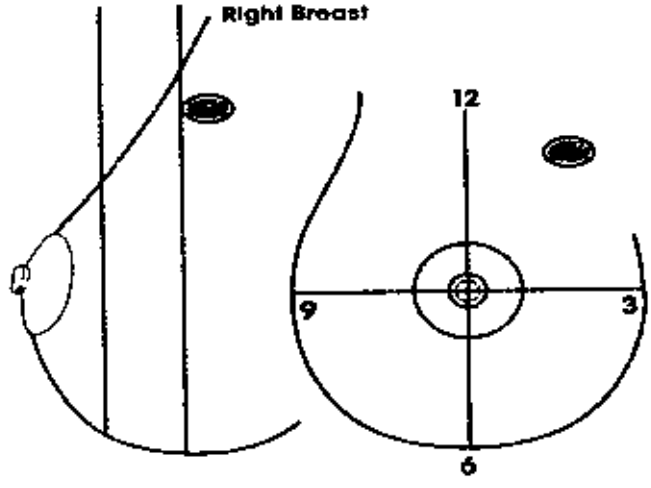
Dear Prof. GamalEmira,

A skin marker was drawn for two tiny rather adherent, 3 mm apart, suspicious solid masses at the upper inner cuadrant (1 O'clock), with the arm elevated in the supine position.

They measure: 5X3 and 3.5X3 mm in diameter and together 8.5X3 mm.

Relation to the Nipple: 10 cm from the nipple.

Depth from skin: 7 mm under the skin.



Prof. Ashraf Selim

N.m

صحتك سعادتك
YOUR HEALTH, YOUR HAPPINESS

Breast Biopsy
[[DOCTYPE:BREAST_BIOPSY]]

38 Mohi El Din Abo El Ezz St., 4th floor, Dokki, Giza, Egypt, TEL.: 02 37 486 100/300/500, FAX: 02 37 49 33 12, www.sohascan.com

صحة سكان
صحة سكان للأشعة والتحاليل
SEHA SCAN & LAB

LHA
إتحاد صحة السيدات
LADIES HEALTH ALLIANCE

MRS. NERMIN AMALI THABET
DATE. 07/07/2024
PROF. GAMAL AMIRA
CODE. 264359

HIGH RESOLUTION ULTRASOUND EXAMINATION OF BOTH BREASTS:

CLINICAL SUMMARY : Bilateral breast lumps.

IMPRESSION :

LEFT BREAST: BIRADS 3 = probably benign.

RIGHT BREAST: BIRADS 4B = moderate suspicion for malignancy.

RECOMMENDATION:-

U/S guided biopsy from the right breast.

Annual screening mammogram starting at the age of 40 years.

Monthly breast self examination. Annual clinical examination.

DETAILED FINDINGS :

TECHNIQUE:

High Resolution Ultrasound with Elastography & Color Doppler.

LEFT BREAST:

- Preserved fibroglandular pattern.
- An oval circumscribed homogenously hypoechoic solid mass with parallel orientation is seen at the upper outer quadrant (1 O'clock). It measures 8X5 mm in diameter. It shows mild posterior enhancement with no vascularity by Power Doppler likely a fibroadenoma. No associated calcifications detected within.
- Normal skin/nipple/areolar complex.
- Preserved axillary lymph nodes.

RIGHT BREAST:

- Preserved fibroglandular pattern.
- Two tiny ill-defined solid nodules are observed at the upper inner quadrant one above the other separated by approximately 3 mm. They measure 5X3 and 3.5X3 mm in diameter. No calcifications or high vascularity detected within.
- Normal skin/nipple/areolar complex.
- Few axillary lymph nodes with thin uniform cortex and preserved fatty hila are noted.

Prof. Ashraf Selim

N.m

صحتك سعادتك
YOUR HEALTH. YOUR HAPPINESS

Sonography
[[DOCTYPE:SONOGRAPHY]]

38 Mohi El Din Abo El Ezz St., 4th floor, Dokki, Giza, Egypt, TEL.: 02 37 486 100/300/500, FAX: 02 37 49 33 12, www.sehascan.com

PATIENT NAME: NERMEEN AMALY THABET HANA
GENDER: Female
DATE : 01-Aug-2024 04:25 PM

PATIENT ID: 10124246439
AGE: 37 y
ORDER: 2970895

TARGET ULTRASOUND EXAMINATION ON LEFT BREAST

INDICATIONS:

- Second look US at the left UIQ tiny enhancing nodule seen in CESM
- Right breast pathologically proved DCIS

TECHNIQUE:

- Ultrasound examination

REPORT:

Left 9-10:00 well-circumscribed hypoechoic solid lesion with lobulated outline along with ductal extension measuring 6.9x6.7 mm with anterior ductal extension reaching 13 mm, 22 mm from the nipple, showing no vascularity with color doppler study
Right breast showed 1-3:00 multiple small hypoechoic well-circumscribed nodules ?? Intraductal extending from the index lesion toward the nipple averaging 4.5 mm

OPINION:

- **Left 9-10:00 probably benign looking solid lesion ?? Intraductal. Regarding contralateral side, for tissue biopsy to exclude atypical features (BIRADS 3).**
- **Right UIQ multiple indeterminate nodules ?? Intraductal. ?? Satellites ?? Other. For functional study to exclude multicentricity (BIRADS 3)**

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Yasmin Nada, MSc **01-Aug-2024 05:16 PM**

www.Baheya.org
16602
٨ شارع ملوية أول شارع الهرم محور دارى . الجيزة
٠ الحي الأول الشيخ زايد امام كلية الهندسة

Sonography
[[DOCTYPE:SONOGRAPHY]]

مبادرة رئيس الجمهورية لدعم صحة المرأة المصرية

صحة

MDT FORM 8 / 8 / 2024 GB

Name	نرمين امال ثابت	MRN	٢٩٠٧١٢٤
		ID	٢٨٧٠٧٢٠٢٠٠٠٧١
Age	37 yrs	Phone	01206991774
Menopausal status	premenopausal	Marital status	married
NRS SCORE		Family history	uncle & grandfather gastric cancer
BMI		REASON OF MDT	
CO morbidity	Asthma, 2CS		

Clinical Data

BX: DCIS intermediate grade L

ER: 8/8 +ve	PR:	HER 2:	KI67:



.8 x .1 cm

Sonomammography: Rt breast UIQ irregular lesion 20'clock

Mets Workup: Chest X ray free

PAMS free

T1N0M0

Recommendations: 9 pt presented with rt breast mass for 2 months:

CESM: Rt breast suspicious nodular enhancement towards retroareolar region, Rt axillary LN & thoracic cortical thickness

MDT OUTCOME & PLAN: For 2 wires localization → WLE & SLNB

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 28/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Right breast mass.

Nature of Specimen:
Tru-cut biopsy collected 9/7/2024.

Technique:
-Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP [illegible]hromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2) ,P63(4A4) and Calponin (EP798Y).

Results:
* IPT (Positive control showed appropriate reactivity):
-P63 and Calponin:Positive in myoepithelial cells outside the ducts.
- ER: Strongly positive (8/8).

Conclusion:
Right breast mass, Tru-cut biopsy:
Findings are compatible with intermeiate grade DCIS positive for ER (8/8).
Comment:No invasion could be detected within the limits of biopsy of biopsy received.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Women's Health Outreach Program

Name: Nermine Amali Sabet Hana
Date: 08-08-2024

Targeted US to Right breast

Clinical Summary: Right sided pathologically proven DCIS

Detailed Findings

Right breast

- **2:00 three small hypoechoic solid nodules each is of average size 3 to 5mm maximum diameter and separated by 3mm apart those are seen 7mm deep to skin. The nearest one to nipple is located at zone b about 28 mm far from nipple. Singie prominent retroareolar duct of 19mm length and reaching 2mm in calibre with no inspissated secretion.**
- No cystic lesions.
- Normal skin/nipple/areolar complex
- No pathological axillary LNs.

Conclusion:

- **Right breast:** Pathological y proven DICS (BI-RADS 6)
- **Left breast:** BIRADS 3 benign solid lesion
-

Much Obliged,
Dr. Rehab Elsheikh , MD

Sonography
[[DOCTYPE:SONOGRAPHY]]

توضيح امام

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name. Nermin Amali Thabet Hanna	Date : 27/06/2024
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID 28707100104301	Pat. ID: 12175

Combined bilateral breast mammography and complementary ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UIQ irregular mass lesion, of equal density.
- Left breast no suspicious microcalcifications or spiculated masses.
- Complementary ultrasound revealed right breast irregular hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 2 o'clock position measuring 0.8x0.4 mm.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Nonspecific bilateral axillary LNs.

Opinion:

- Right breast UIQ irregular mass lesion ?? benign looking ?? indeterminate for further assessment and close follow up / histopathological correlation if clinically warranted (BIRADS 3)
- Benign findings left breast (BIRADS 2)

Best regards,
Dr/Asmaa Zeinhom, Msc

Not Approved

Mammogram
[[DOCTYPE:MAMMOGRAM]]

ده التقرير القديم

B.HEYA
Radiology Department

بهية
قسم الأشعة التشخيصية

PATIENT NAME: NERMEEN AMALY THABET HANA
GENDER: Female
DATE : 31-Jul-2024 03:52 PM

PATIENT ID: 10124246439
AGE: 37 y
ORDER: 2969012,2969504

CONTRAST ENHANCED SPECTRAL MAMMOGRAPHY AND COMPLEMENTARY ULTRASOUND

INDICATION:
- right breast DCIS

TECHNIQUE:
- Contrast Enhanced Spectral Mammography CC and MLO views
- Complementary ultrasound

REPORT:
- Bilateral dense fibroglandular densities (ACR C).
- Bilateral scattered homogeneously enhancing nodules likely adenotic
- Right upper inner deeply seated heterogeneously enhancing nodules arranged in linear pattern largest measured 3 mm with anterior enhancing nodular extension towards retroareolar region confirmed by ultrasound
- Left UIQ homogeneously enhancing nodule corresponding by ultrasound to well dfeined hypoechoic lesion
- Normal skin thickness and contour.
- Normal nipple areola complexes.
- Right axillary lymph node with increased cortical thickness reaching 4 mm.

OPINION:
-Right breast suspicious nodular enhancement towards retroareolar region (BiRADS 6).
- No left breast pathological mass or nor mass enahncement with UIQ small adenoma (BIRADS 3)

Reported By,

Dr. Amira Atef Hussain, MSc **31-Jul-2024 04:32 PM**

www.Baheya.org
16602
مركز بهية الهرم
٨ شارع طوبة أول شارع الهرم بجوار دلوي ، الجيزة
مستشفى بهية الشيخ زايد
١٠ الحي الأول الشيخ زايد امام كلية الهندسة

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
دارة الاشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 28/07/2024
Exam : Ultrasound ABDOMEN& PELVIS	Age : 37 Year(s)
Medical ID: 2407/2024	Pat. ID: 12175

ABDOMINO-PELVIC ULTRASOUND EXAMINATION REVEALED:

Liver: -

- *Mildly enlarged in size (about 18 cm at the right MCL), smooth outline, bright echopattern. No intra hepatic biliary radicle dilatation.*
- *Portal vein is not dilated and patent .*

Gall Bladder: -

- *Average size , normal wall thickness, no stones.*
- *CBD is not dilated.*

Spleen: *Average size (About 12 cm), displaying homogenous echopattern.*

Kidneys: -

- *Are of normal size and site , normal echogenicity , no stone or backpressure changes , no solid or cystic lesion.*
- *Both ureters are noted dilated.*

URINARY BLADDER*: Normal filling. Normal wall thickness, no stones or diverticulae or solid lesion detected.*

Uterus: -:

- *anti-verted, average sized uterus. Average endometrial thickness, no focal lesions.*

Ovaries: - *both are average in size with no dominant follicle could be detected.*

- ***No free fluid in the Douglas pouch.***
- *Para-aortic and pancreatic area: no gross lesion*

Opinion:

- *Bright hepatomegaly likely fatty.*
- *Otherwise; normal rest of study.*

Best Regards,
Dr. Maataz Adel, MSc.

Signature Dr . Moataz [illegible]im
PM 2:05 2024/07/28

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Date : 28/7/2024	Patient Name: Nermin Amali Thabet Hanna
Age : [illegible](s)	Exam : X-RAY CHEST
Pat. ID: 12175	Medical ID: 247/2024

Plain X-ray Chest (PA view) revealed:

- *Clear both lung fields.*
- *Clear both costo-phrenic angles.*
- *Normal cardiac size & configuration.*

Best regards

Dr. Moataz Adel ,MSc.

Signature Dr . Moataz Adel Ebranim
28/07/2024 2:09 PM

Ultrasound
[[DOCTYPE:ULTRASOUND]]

Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

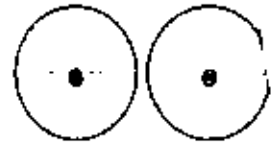
MDT form

A-Patient ID:
1- Name : ترمين امقى ثابت حنا Sex: Female
3- Medical number: 83221549
Old Med : 2407-2024

2-ID number: 28707100104301

Patient Age : 37 Y

MDT



Family history	Family History +ve	Phone	Phone 01200991774	BMI	BMI	REASON OF MDT	REASON OF MDT REMOT
marital status	Marital Status MARRIED	NRS Score	NRS Score	GROUP	GROUP a	Date of MDT	Date Of MDT 08 / 08 / 2024
Co morbidity	Co Morbidity ashtma , 2 cs	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
rt breast UIQ irregular lesion 2 oclock 0.8 x0.1 mm

Mets Workup :
chest xray : free
PAUS : free

BX	ER	PR	Her 2 Neu	SISH	KI67
DCIS BX COMMENT:	8/8 ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT :	

T	N	M

Sonomammography
cesm: rt breast susupicous nodular enahncement towards retroareolar region
rt axillary LN with increase cortical thickness

PATIENT HISTORY
target u/s FOR Rt UIQ multiple indeterminate nodules to exclude intraductal extension to the nipple

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY د/	RADIOLOGY	RADIOLOGY د/
SURGERY	SURGERY د/	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

مبـــادرة رئيس الجمهورية
لدعـم صحة المرأة المصرية

الست المصرية هي صحة مصر

MDT FORM / / 20 G

Name	نيرمين احمد ثابت	MRN	٨٣٢٢١٥٤٩
		ID	٠١٢٨٨٨٧٣١١
Age	37	Phone	٠١٢٠٠٩٩١٧٧٤
Menopausal status		Marital status	
NRS SCORE		Family history	
BMI		REASON OF MDT	
CO morbidity	Asthma		

Clinical Data

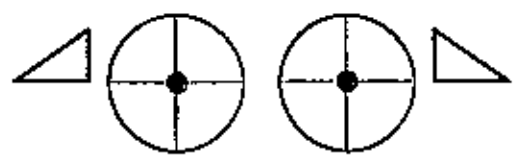
BX: OCIs

ER:	PR:	HER 2:	Ki67:
8/8			

Sonomammography:

Mets Workup:

R L



T N M

Recommendations:

Rt BCS + SNIB Recieved Port and on adj Tam for 5yrs

MRI Pelvis shows Rt adnexal Ovarian mass

MDT OUTCOME & PLAN

SIGNATURE

MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY	Nutrition	Psychiatric

MDT Form
[[DOCTYPE:MDT_FORM]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002824
Patient No.	: 28707100104301	Request Date	: 16/02/2025
Path No.	: 668/25	Verification Date	: 06/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
VAGINAL BLEEDING

Nature of specimen
D&C

Gross specimen

Multiple fragmented hemorrhagic pieces measured1x1 cm, totally submitted

Microscopic examination
histopathlgical examination of sections prepared from the endometrial curettings received reveals late secretory glands with exhausted secretory activity. The glands are tortuous and lined by low cuboidal epithelium. The stroma shows late decidualization around well formed spiral arterioles and in between most of the glands. There is no evidence of endometritis or hyperplasia in sections examined.

Diagnosis
Vaginal bleeding.. D&C :
EXAGERATED SECRETORY ENDOMERIUM

Signature
Prof. Dr. Amal Hareedy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

CAP ACCREDITED
غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

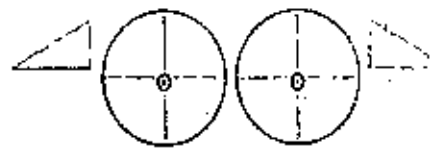
Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان

MDT form

A-Patient ID:
1- Name : ترمين امانى ثابت حنا
Sex: Female
3- Medical number: 83221549
Old Med : 2407-2024

2-ID number: 28707100104301
Patient Age : 37 Y



Family history	Family history +Ve	Phone	01200991774	BMI		REASON OF MDT	REASON OF MDT
marital status	Marital Status MARRIED	NRS Score		GROUP	GROUP D	Date of MDT	Date of MDT 2025-05-13
Co morbidity	Co Morbidity asthma , 2 cs	Menopausal status	Menopausal Status PREMENOPAUSAL	Initial STAGE	Initial STAGE		

Clinical data

Breast Imaging :
rt breast UIQ irregular lesion 2 oclock 0.8 x0.1 mm

Mets Workup :
chest x-ray : free
PAUS : free

BX	ER	PR	Her 2 Neu	SISH	KI67
DCIS	8/8				
BX COMMENT:	ER COMMENT:	PR COMMENT:	Her 2 Neu COMMENT:	SISH COMMENT :	

T	N	M

Sonomammography
cesm: rt breast suspicious nodular enhancement towards retroareolar region
rt axillary LN with increase cortical thickness

PATIENT HISTORY
target u/s FOR Rt UIQ multiple indeterminate nodules to exclude intraductal extension to the nipple
RT BCS+SLNB: free all resected margin (1 mm only)
free LN 0/3
on adj tam for 5 years
MRI pelvis show rt adenxal (ovarian) mass
PELVIC MRI RT ADENEXAL LESION 6 X6X 10 CM WITH NO ENHANCED SOFT TISSUE COMP.
CA125 NORMAL

MDT OUTCOME & PLAN

MDT OUTCOME & PLAN
FOLLOW UP EVERY 3 MS BY PAUS CYST EXCISION IF SYMPTOMATIC

SIGNATURE

MEDICAL ONCOLOGY	MEDICAL ONCOLOGY	RADIOLOGY	RADIOLOGY
SURGERY	SURGERY	PATHOLOGY	PATHOLOGY
Psychiatric	Psychiatric	Nutrition	Nutrition

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13001643
Patient No.	: 28707100104301	Request Date	: 14/08/2024
Path No.	: 2942/24	Verification Date	: 02/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - مباشره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical Data:
Right breast suspicious nodular lesion.

Nature of the specimen:
Right wide local excision and sentenil lymph node biopsy specimens , collected at 14/8/2024.

Gross Examination:
Three specimens were recieved :
-Sentenil lymph node biopsy:Multiple fibrofatty tissue peices collectivelly measured 3.5x3.5 cm , enclosing 3 nodules up to 2x1x0.5cm.
-Extra superficial margin:A skin elipse measured 3x1.5 cm with subcutis measured 0.5 cm thick.It showed an incision measured 2.5 cm long.Also received multiple seperate fibrofatty tissue peices measured 3x1x0.5 cm , multisected and totally submitted.
-Right wide local excision specimen measured 6x5x2 cm with no covering skin elipse .
- Tumor extent
o Number: An ill defined firm grey'sh area (wired) , multisected and totally submitted.
o Size: 2x1x0.5 cm.
- Distance from margins:0.2 cm from superior , medial and lateral margins , 0.1 cm from inferior , 2.5 cm from deep and less than 0.1 cm from superficial margin.
- Frozen section:
-Free SLNs(0/3).
-Suspicious superficial margin for DCIS.
-Free other margins.
- Gross key:
1-4)Grey white area (totally submitted).
5-Medial margin.
6-Lateral margin.

Verified by :
dr.esraa magdy

CAP ACCREDITED
غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذة / نرمين امالي ثابت حنا	Request No.	: 13002824
Patient No.	: 28707100104301	Request Date	: 16/02/2025
Path No.	: 668/25	Verification Date	: 06/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
VAGINAL BLEEDING

Nature of specimen
D&C

Gross specimen

Multiple fragmented hemorrhagic pieces measured1x1 cm, totally submitted

Microscopic examination
histopathological examination of sections prepared from the endometrial curettings received reveals late secretory glands w :h exhausted secretory activity. The glands are tortuous and lined by low cuboidal epithelium. The stroma shows late decidualization around well formed spiral arterioles and in between most of the glands. There is no evidence of endometritis or hyperplasia in sections examined.

Diagnosis
Vaginal bleeding.. D&C :
EXAGERATED SECRETORY ENDOMERIUM

Signature
Prof Dr. Amal Hareedy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

National ID – Front
[[DOCTYPE:ID_FRONT]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13001643
Patient No.	: 28707100 04301	Request Date	: 14/08/2024
Path No.	: 2942/24	Verification Date	: 02/09/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mchamed youss

Pathology Report

7-Superior margins.
8-Inferior margin.
9-Superficial margin.
10-Deep margin.
11,12-Extra superficial margin.
13-Sentenil lymph nodes.

Microscopic examination:
- Histological type:Ductal carcinoma insitu , low grade.
-Intraductal papilloma with pseudoinvasion.
-Columanr cell change and ductal hyperplasia.
-No evidence of invasion in sections examined.
- Angioinvasion: Not detected .
- Lymph node involvement:Free SLNs (0/3)
- Resection Margins: Free all resected margins.

Diagnosis:
Right breast suspicious nodular lesion, wide local excision and sentenil specimens:
-Ducttal carcinoma insitu , low grade.
-Intraductal papilloma.
-Columnar cell changes and ductal hyperplasia.
No evidence of invasion in sections examined.
-Free all resected margins
-Free SLNs (0/3)
-Stage : pTis , pN0

Prof.Gamal Abdelatty
Dr.Menna Nabil

Verified by :
dr.esraa magdy

CAP ACCREDITED
غرفة رقم (4) – الدور الاول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Curniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصحة والسكان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 27/06/2024
Exam : MAMMOGRAPHY BOTH	Age : 37 Year(s)
Medical ID: 28707100104301	Pat. ID: 12175

Combined bilateral breast mammography and complementary ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular density (ACR B)
- Right breast shows UIQ irregular mass lesion, of equal density.
- Left breast no suspicious microcalcifications or spiculated masses.
- Complementary ultrasound revealed right breast irregular hypoechoic mass lesion, with subtle vascularity detected within on color Doppler, seen at 2 o'clock position measuring 0.8x0.4 mm.
- Normal skin thickness, and nipple-areola complex of the left breast.
- Nonspecific bilateral axillary LNs.

Opinion:

- Right breast UIQ irregular mass lesion ?? benign looking ?? indeterminate for further assessment and close follow up / histopathological correlation if clinically warranted (BIRADS 3)
- Benign findings eft breast (BIRADS 2)

Best regards,
Dr/Asmaa Zeinhom, Msc

Not Approved

Mammogram
[[DOCTYPE:MAMMOGRAM]]

Patient Name	: الأستاذه / ترمين امالي ثابت حنا	Request No.	: 13002824
Patient No.	: 28707100104301	Request Date	: 16/02/2025
Path No.	: 668/25	Verification Date	: 06/03/2025
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc	:	Registered by	: .marwa mohamed youss

Pathology Report

Clinical data
VAGINAL BLEEDING

Nature of specimen
D&C

Gross specimen

Multiple fragmented hemorrhagic pieces measured1x1 cm, totally submitted

Microscopic examination
histopathoigical examination of sections prepared from the endometrial curettings received reveals late secretory glands with exhausted secretory activity. The glands are tortuous and lined by low cuboidal epithelium. The stroma shows late decidualization around well formed spiral arterioles and in between most of the glands. There is no evidence of endometritis or hyperplasia in sections examined.

Diagnosis
Vaginal bleeding.. D&C :
EXAGERATED SECRETORY ENDOMERIUM

Signature
Prof Dr. Amal Hareedy
Dr.Heba Hassan

Verified by :
Dr.Heba Hasan

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 28/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patien:
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMM[illegible]STOCHEMISTRY REPORT

Clinical Data:
Right breast mass.

Nature of Specimen:
Tru-cut biopsy collected 9/7/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2) ,P63(4A4) and Calponin (EP798Y).

Results:
* IPT (Positive control showed appropriate reactivity):
-P63 and Calponin:Positive in myoepithelial cells outside the ducts.
- ER: Strongly positive (8/8).

Conclusion:
Right breast mass, Tru-cut biopsy:
Findings are compatible with intermediate grade DCIS positive for ER (8/8).
Comment:No invasion could be detected within the limits of biopsy of biopsy received.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

Ministry Of Health
Specialized Medical Centers
Dar El Salam Cancer Hospital
Radiology Department

وزارة الصـــــحة والسكـــان
أمانة المراكز الطبية المتخصصة
مستشفى أورام دار السلام
ادارة الاشعة

Patient Name: Nermin Amali Thabet Hanna	Date : 07/06/2023
Exam : MAMMOGRAPHY BOTH	Age : 36 Year(s)
Medical ID: 1734	Pat. ID: 12175

Full Field Digital bilateral breast mammography & Complementary Ultrasound

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Ultrasound examination using linear probe 6-12MHz

Report:

- ACR C: heterogeneously dense parenchyma.
- No suspicious micro calcifications or architectural distortion detected.
- No spiculated mass
- Left breast UIQ multiple small well defined hypoechoic nodules area seen at 9o'clcok, 10o'clcok,and 11 o'clcok measuring about 5x5mm, 4x4mm and 9x6mm respectively noted>>> seen with possible ductal communication. It is seen at 3.2 cm distance form the nipple (?? small papilloma ?? Small fibroadenoma)
- Prominent left retro-areolar ducts
- Normal skin thickness, and nipple-areola complex
- Nonspecific axillary LN.

Opinion:

- Bilateral mammary fibroadenosis.
- Left breast UIQ probably-benign mass as described (BIRADS 3), for U/S follow up after 6 months

- Signature Dr. Nehal Hamdy
- 07/06/2023 5:01 PM

Mammogram
[[DOCTYPE:MAMMOGRAM]]

مركز أورام
دار السلام
قبل سابقا

وزارة الصحه و السكان
أمانة المراكز الطبية المتخصصه
مركز أورام دار السلام
"مرمل سابقا"

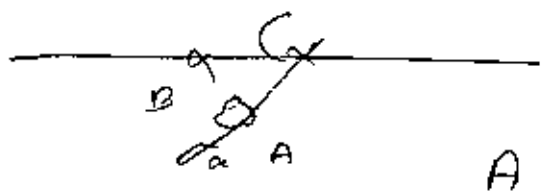
Name. Nermin Amali Thabet	*37Y*	*F*
Date: 13/08/2024		

Wire localization for right breast mass:

- *Under local anesthesia, antiseptic condition and ultrasound guidance.*
  - *Ultrasound guided wire localization was inserted into the right breast 2 o'clock lesion*
  - *Whole distance of the wire till the tip: 2.1 cm*
  - *Perpendicular distance from skin to tip: 1 cm*
  - *Distance from perpendicular distance to skin entry: 1.8 cm.*
  - *Skin mark (X) was placed over the lesion*

Best Regards,

Dr. Ahmed M. Rozeka, Msc
Specialist Radiologist



A = 2.1 cm.
B = 1 cm
C = 1.8 cm

Sonography
[[DOCTYPE:SONOGRAPHY]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 15/07/2024
Sex	: Female	Financial Category	: اورام دار لسلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Mrs.Mayada Hegazy

Pathology Report

Clinical data:
Right breast mass.

Nature of specimen:
Tru cut biopsy, collected at 9/7/2024.

Gross examination:
Three greyish white tissue cores measured up to 1cm , totally submitted.

Microscopic examination:
Examinations revealed cores of breast tissue with many foci of intraductal proliferation showed monotonus cells with moderate pleomorphism and hyperchromasia.

Diagnosis:
Right breast mass, tru cut b opsy:
Findings are compatible with intermediate grade DCIS.
For further IHC (ER,P63,Calponin) to detect any foci of invasion.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Pathology Report
[[DOCTYPE:PATHOLOGY_REPORT]]

Patient Name	: الأستاذه / نرمين امالي ثابت حنا	Request No.	: 13002054
Patient No.	: 28707100104301	Request Date	: 10/07/2024
Path No.	: 2321/24	Verification Date	: 28/07/2024
Sex	: Female	Financial Category	: اورام دار السلام - مبادره
Age	: 37 Y	Department	: Out Patient
Referreing Doc.	:	IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:
Right breast mass.

Nature of Specimen:
Tru-cut biopsy collected 9/7/2024.

Technique:
- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2) ,P63(4A4) and Calponin (EP798Y).

Results:
* IPT (Positive control showed appropriate reactivity):
-P63 and Calponin:Positive in myoepithelial cells outside the ducts.
- ER: Strongly positive (8/8).

Conclusion:
Right breast mass, Tru-cut biopsy:
Findings are compatible with intermediate grade DCIS positive for ER (8/8).
Comment:No invasion could be detected within the limits of biopsy received.

Dr.Marwa Mahmoud
Dr.Esraa Magdy

Verified by :
dr.esraa magdy

غرفة رقم (4) – الدور الأول – مبنى رقم (987) – كورنيش النيل – مصر القديمة – القاهرة – مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Immunohistochemistry
[[DOCTYPE:IHC]]

B.HEYA
Radiology Department

بـ.ـيـة
قسم الأشعة التشخيصية

PATIENT NAME: NERMEEN AMALY THABET HANA
GENDER: Female
DATE : 01-Aug-2024 04:25 PM

PATIENT ID: 10124246439
AGE: 37 y
ORDER: 2970895

TARGET ULTRASOUND EXAMINATION ON LEFT BREAST

INDICATIONS:

- Second look US at the left UIQ tiny enhancing nodule seen in.CESM
- Right breast pathologically proved DCIS

TECHNIQUE:

- Ultrasound examination

REPORT:

Left 9-10:00 well-circusmcribed hypoechoic solid lesion with lobulated outline along with ductal extension measuring 6.9x6.7 mm with anterior ductal extension reaching 13 mm, 22 mm from the nipple, showing no vascularity with color doppler study
Right breast showed 1-3:00 multiple small hypoechoic well-circusmcribed nodules ?? Intraductal extending from the index lesion toward the nipple averaging 4.5 mm

OPINION:

- **Left 9-10:00 probably benign looking solid lesion ?? Intraductal. Regarding contralateral side, for tissue biopsy to exclude atypical features (BIRADS 3).**
- **Right UIQ multiple indeterminate nodules ?? Intraductal. ?? Satellites ?? Other. For functional study to exclude multicentricity (BIRADS 3)**

RECOMMENDATION:

Referral to clinic

Reported By,

Dr. Yasmin Nada, MSc **01-Aug-2024 05:16 PM**

www.Baheya.org
16602
Sonography
[[DOCTYPE:SONOGRAPHY]]